

India's trusted Health Test @Home Service



Booking ID : 15711877853

Sample Collection Date : 01/Nov/2025

Radheshyam B Thakkar

Male, 42 Yrs

A Comprehensive Health Analysis Report

AI Based Personalized Report for You



INDIA'S FIRST & ONLY CREDIBILITY CHECK FOR YOUR LAB REPORT

Check the authenticity of your lab report with machine data

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HEALTH ANALYSIS

Personalized Summary & Vital Parameters

Radheshyam B Thakkar

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Radheshyam B Thakkar,

Congratulations, We have successfully completed your health diagnosis. This is a big step towards staying on top of your health and identify potential to improve!

10 Vital Health Parameters of a Human Body Ecosystem

Below are the health parameters which require routine checkups for primary healthcare. The view also includes *personalised information* depending on the tests you have taken.

Your Health Score

72

Out of 100

*Calculated from test reports



Thyroid Function

Thyroid Stimulating Hormone (TSH) - Ultrasensitive : 2.03 μ IU/ml

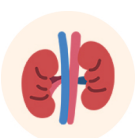
● Everything looks good



Cholesterol Total

144.5 mg/dl

● Everything looks good



Kidney Function

Serum Creatinine : 1.03 mg/dl

● Everything looks good



Vitamin D

28.75 ng/ml

● Concern



HbA1c

6 %

● Concern



Vitamin B12

316 pg/ml

● Everything looks good



Liver Function

Alanine Aminotransferase (ALT/SGPT) : 7 U/L

● Everything looks good



Calcium Total

8.4 mg/dl

● Everything looks good



Iron studies

Serum Iron : 15 ug/dl

● Concern



Complete Hemogram

Haemoglobin (HB) : 8.7 g/dL

● Concern





New Features

Report Summary

Radheshyam B Thakkar

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Understanding laboratory reports can be complex, often leading to unwarranted anxiety.

At Healthians, we understand that you shouldn't have to rely on a Google search to decipher your own health report. That's why we offer comprehensive summaries that are easy to understand.

Summary of Health Test Results

Dear Radheshyam,

Thank you for sharing your health test results. It's important to remember that test values can fluctuate for various reasons, and many factors can influence your overall health. Based on your recent tests, there are a few areas that may need attention, but please know that with the right approach, improvements can be made.

Your vitamin D levels are slightly below the normal range, which is quite common and can often be addressed through dietary changes or increased sunlight exposure. Additionally, some of your blood counts indicate that your body may be experiencing some stress or inflammation, which can be managed with lifestyle adjustments.

Your hemoglobin and iron levels are lower than what is typically expected, which can sometimes lead to feelings of fatigue. This is often linked to dietary intake, and focusing on iron-rich foods could be beneficial.

Lastly, your liver function tests suggest that there may be some imbalance, but this is something that can often be improved with dietary modifications and lifestyle changes.

Suggestions for Improvement

1.

Enhance Vitamin D Levels

: Consider incorporating more vitamin D-rich foods into your diet, such as fatty fish, fortified dairy products, and egg yolks. Additionally, spending some time outdoors in sunlight can help boost your levels naturally.

2.

Focus on Nutrition

: To address your hemoglobin and iron levels, try to include more iron-rich foods like lean meats, beans, lentils, and leafy green vegetables in your meals. Pairing these with vitamin C-rich foods can enhance iron absorption.

3.

Stay Hydrated and Active

: Ensure you are drinking plenty of water and engaging in regular physical activity. This can help improve your overall blood counts and support your liver function.

4.

Monitor Your Diet

: Pay attention to your overall dietary habits. A balanced diet rich in whole foods, fruits, vegetables, and healthy fats can support your liver health and overall well-being.

Remember, small changes can lead to significant improvements over time. Taking proactive steps towards your health can make a positive difference.

Patient Name	: Radheshyam B Thakkar	Barcode	: E3959015
Age/Gender	: 42Y 0M 0D /Male	Sample Collected On	: 01/Nov/2025 10:34AM
Order Id	: 15711877853	Sample Received On	: 01/Nov/2025 03:37PM
Referred By	: Self	Report Generated On	: 01/Nov/2025 05:35PM
Customer Since	: 01/Nov/2025	Sample Temperature	: Maintained ✓
Sample Type	: Whole Blood EDTA	Report Status	: Final Report

DEPARTMENT OF BIOCHEMISTRY HBA1C

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Test Name	Value	Unit	Bio. Ref Interval
HbA1c - Glycosylated Hemoglobin			
Hba1c (Glycosylated Hemoglobin)	6.00	%	4.2 - 5.7
Method: HPLC			
Machine: Tosoh G8			
Average Estimated Glucose - plasma	125.50		
Method: Calculated			

INTERPRETATION:

AS PER AMERICAN DIABETES ASSOCIATION (ADA):

REFERENCE GROUP

Non diabetic
At Risk (Prediabetes)
Diagnosing Diabetes

GLYCOSYLATED HEMOGLOBIN (HBA1c) in %

<5.7
5.7 – 6.4
≥ 6.5

Age > 19 Years

Goals of Therapy: < 7.0
Actions Suggested: >8.0

Age < 19 Years

Goal of therapy: <7.5

Therapeutic goals for glycemic control

REMARKS

- HbA1c is used for monitoring diabetic control. It reflects the mean plasma glucose over three months
 - HbA1c may be falsely low in diabetics with hemolytic disease. In these individuals a plasma fructosamine level may be used which evaluates diabetes over 15 days.
 - Inappropriately low HbA1c values may be reported due to hemolysis, recent blood transfusion, acute blood loss, hypertriglyceridemia, chronic liver disease. Drugs like dapsone, ribavirin, antiretroviral drugs, trimethoprim, may also cause interference with estimation of HbA1c, causing falsely low values.
 - HbA1c may be increased in patients with polycythemia or post-splenectomy.
 - Inappropriately higher values of HbA1c may be caused due to iron deficiency, vitamin B12 deficiency, alcohol intake, uremia, hyperbilirubinemia and large doses of aspirin.
 - Trends in HbA1c are a better indicator of diabetic control than a solitary test. 7. Any sample with >15% HbA1c should be suspected of having a hemoglobin variant, especially in a non-diabetic patient. Similarly, below 4% should prompt additional studies to determine the possible presence of variant hemoglobin.
 - HbA1c target in pregnancy is to attain level <6 % .
 - HbA1c target in paediatric age group is to attain level < 7.5 %.
- Method : Ion-exchange high-performance liquid chromatography (HPLC).
Reference : American Diabetes Associations. Standards of Medical Care in Diabetes 2023



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Page 1 of 17

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Order Id	: 15711877853	Sample Received On	: 01/Nov/2025 03:35PM	
Referred By	: Self	Report Generated On	: 01/Nov/2025 03:51PM	
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Sample Type	: Flouride Plasma	Report Status	: Final Report	

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Test Name	Value	Unit	Bio. Ref Interval
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Fasting Blood Sugar

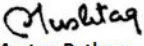
Glucose, Fasting	80.79	mg/dl	70 - 100
Method: Enzymatic (Hexokinase/G-6-PDH)			
Machine: Abbott ALINITY CI			

American Diabetes Association Reference Range :

Normal : < 100 mg/dl
 Impaired fasting glucose(Prediabetes) : 100 - 126 mg/dl
 Diabetes : >= 126 mg/dl

Conditions that can result in an elevated blood glucose level include: Acromegaly, Acute stress (response to trauma, heart attack, and stroke for instance), Chronic kidney disease, Cushing syndrome, Excessive consumption of food, Hyperthyroidism, Pancreatitis

A low level of glucose may indicate hypoglycemia, a condition characterized by a drop in blood glucose to a level where first it causes nervous system symptoms (sweating, palpitations, hunger, trembling, and anxiety), then begins to affect the brain (causing confusion, hallucinations, blurred vision, and sometimes even coma and death). A low blood glucose level (hypoglycemia) may be seen with: Adrenal insufficiency, Drinking excessive alcohol, Severe liver disease, Hypopituitarism, Hypothyroidism, Severe infections, Severe heart failure, Chronic kidney (renal) failure, Insulin overdose, Tumors that produce insulin (insulinomas), Starvation.


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Referred By	: Self	Report Generated On	: 01/Nov/2025 05:30PM	
Customer Since	: 01/Nov/2025	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

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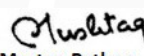
Test Name	Value	Unit	Bio. Ref Interval
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Rheumatoid Factor (RA) - Quantitative - Serum

RHEUMATOID FACTOR	< 20.0	IU/mL	<30
Method: Immunoturbidimetry			
Machine: Abbott ALINITY CI			

The rheumatoid factor (RF) test is primarily used to help diagnose rheumatoid arthritis (RA) and to help distinguish RA from other forms of arthritis or other conditions that cause similar symptoms. A cyclic citrullinated peptide (CCP) antibody test can help diagnose RA in someone who has joint inflammation with symptoms that suggest but do not yet meet the criteria of RA and may be ordered along with RF or if the RF result is negative. The RF test must be interpreted in conjunction with a person's symptoms and clinical history. A negative RF test does not rule out RA.

About 20% of people with RA will have very low levels of or no detectable RF. In these cases, a CCP antibody test may be positive and used to confirm RA. Positive RF test results may also be seen in 1-5% of healthy people and in some people with conditions such as: Sjogren syndrome, scleroderma, systemic lupus erythematosus (lupus), sarcoidosis, endocarditis, tuberculosis, syphilis, HIV/AIDS, hepatitis, infectious mononucleosis, cancers such as leukemia and multiple myeloma, parasitic infection, or disease of the liver, lung or kidney.


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Referred By : Self
Customer Since : 01/Nov/2025
Sample Type : Serum

Barcode : E3959015
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Report Status : Final Report



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Test Name	Value	Unit	Bio. Ref Interval
Liver Function Test (LFT)			
Serum Bilirubin, (Total) Method: Diazo Machine: Abbott ALINITY CI	0.40	mg/dl	0.3 - 1.2
Serum Bilirubin, (Direct) Method: Diazo Machine: Abbott ALINITY CI	0.23	mg/dl	0.0 - 0.5
Serum Bilirubin, (Indirect) Method: Calculated	0.17	mg/dl	0.0 - 0.8
Aspartate Aminotransferase (AST/SGOT) Method: Enzymatic Machine: Abbott ALINITY CI	16.00	U/L	5 - 34
Alanine Aminotransferase (ALT/SGPT) Method: Enzymatic Machine: Abbott ALINITY CI	7	U/L	0 - 55
Alkaline Phosphatase (ALP) Method: Para-nitrophenyl Phosphate Machine: Abbott ALINITY CI	117.00	U/L	40 - 150
Gamma Glutamyl Transferase (GGT) Method: L-Gamma-glutamyl-3-carboxy-4-Nitroanilide substrat Machine: Abbott ALINITY CI	22.0	U/L	12 - 64
Serum Total Protein Method: Biure Machine: Abbott ALINITY CI	7.74	g/dl	6.4 - 8.3
Serum Albumin Method: Bromocresol Green Machine: Abbott ALINITY CI	3.11	g/dl	3.5 - 5.2
Serum Globulin Method: Calculated	4.63	gm/dl	3.0 - 4.2
Albumin/Globulin Ratio Method: Calculated	0.67	Ratio	1.2 - 2.5
SGOT/SGPT Ratio Method: Calculated	2.29	Ratio	0.7 - 1.4

Bilirubin is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Elevated levels results from increased bilirubin production (eg hemolysis and ineffective erythropoiesis); decreased bilirubin excretion (eg; obstruction and hepatitis); and abnormal bilirubin metabolism (eg; hereditary and neonatal jaundice). Conjugated (direct) bilirubin is elevated more than unconjugated (indirect) bilirubin in viral hepatitis; drug reactions, alcoholic liver disease conjugated (direct) bilirubin is also elevated more than unconjugated (indirect) bilirubin when there is some kind of blockage of the bile ducts like in Gallstones

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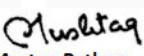
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Test Name	Value	Unit	Bio. Ref Interval
getting into the bile ducts tumors & Scarring of the bile ducts. Increased unconjugated (indirect) bilirubin may be a result of hemolytic or pernicious anemia, transfusion reaction & a common metabolic condition termed Gilbert syndrome.			
AST levels increase in viral hepatitis, blockage of the bile duct ,cirrhosis of the liver, liver cancer, kidney failure, hemolytic anemia, pancreatitis, hemochromatosis. Ast levels may also increase after a heart attck or strenuous activity. ALT is commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health. Elevated ALP levels are seen in Biliary Obstruction, Osteoblastic Bone Tumors, Osteomalacia, Hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, paget's disease, Rickets, Sarcoidosis etc.			
Elevated serum GGT activity can be found in diseases of the liver, Biliary system and pancreas. Conditions that increase serum GGT are obstructive liver disease, high alcohol consumption and use of enzyme-including drugs etc.			
Serum total protein, also known as total protein, is a biochemical test for measuring the total amount of protein in serum..Protein in the plasma is made up of albumin and globulin. Higher-than-normal levels may be due to: Chronic inflammation or infection, including HIV and hepatitis B or C, Multiple myeloma,Waldenstrom's disease. Lower-than-normal levels may be due to: Agammaglobulinemia, Bleeding (hemorrhage), Burns, Glomerulonephritis, Liver disease, Malabsorption, Malnutrition, Nephrotic - Human serum albumin is the most abundant protein in human blood plasma. It is produced in the liver.Albumin constitutes about half of the blood serum protein. Low blood albumin levels (hypoalbuminemia) can be caused by: Liver disease like cirrhosis of the liver, nephrotic syndrome, protein-losing enteropathy, Burns, hemodilution, increased vascular permeability or decreased lymphatic clearance, malnutrition and wasting etc.			



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Page 5 of 17

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Customer Since	: 01/Nov/2025	Sample Temperature	: Maintained ✓
Sample Type	: SERUM	Report Status	: Final Report

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Test Name	Value	Unit	Bio. Ref Interval
Iron study			
Serum Iron	15.0	ug/dl	65 - 175
Method: Ferene			
Machine: Abbott ALINITY CI			
UIBC	338.49	ug/dl	69 -240
Method: Ferene			
Machine: Abbott ALINITY CI			
Serum Total Iron Binding Capacity (TIBC)	353.49	µg/dl	250 - 425
Method: FE+UIBC (saturation with iron)			
Transferrin Saturation %	4.24	%	10 - 50
Method: Calculated			

Iron participates in a variety of vital processes in the body varying from cellular oxidative mechanisms to the transport and delivery of oxygen to body cells. It is a constituent of the oxygen-carrying chromoproteins, haemoglobin and myoglobin, as well as various enzymes, such as cytochrome oxidase and peroxidases.

Serum iron may be increased in hemolytic, megaloblastic and aplastic anemias, and in hemochromatosis acute leukemia, lead poisoning, pyridoxine deficiency, thalassemia, excessive iron therapy, and after repeated transfusions. Drugs causing increased serum iron include chloramphenicol, cisplatin, estrogens (including oral contraceptives), ethanol, iron dextran, and methotrexate. Iron can be decreased in iron-deficiency anemia, acute and chronic infections, carcinoma, nephrotic syndrome hypothyroidism, in protein- calorie malnutrition and after surgery. Diurnal variation is seen in serum iron levels with normal values obtained in the midmorning, low values in midafternoon and very low values near midnight.

TIBC measures the blood's capacity to bind iron with transferrin (TRF). Estrogens and oral contraceptives increase TIBC levels. Asparaginase, chloramphenicol, corticotropin, cortisone, and testosterone decrease the TIBC levels.

Transferrin is the primary plasma iron transport protein, which binds iron strongly at physiological pH. Transferrin is generally only 25% to 30% saturated with iron. The additional amount of iron that can be bound is the unsaturated iron-binding capacity (UIBC). Transferrin saturation represents the number of iron-binding sites that are occupied. It is a better index of iron stores than serum iron alone. Transferrin saturation is decreased in iron deficiency anemia (usually <10% in established deficiency).

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Test Name	Value	Unit	Bio. Ref Interval
Kidney Function Test Advance			
Serum Creatinine	1.03	mg/dl	0.7 - 1.4
Method: Kinetic Alkaline Picrate			
Machine: Abbott ALINITY CI			
GFR, ESTIMATED	93.01	mL/min/1.73m2	
Method: Calculated (CKD-EPI)			
Serum Uric Acid	5.8	mg/dl	3.5 - 7.2
Method: Uricase			
Machine: Abbott ALINITY CI			
Serum Calcium	8.4	mg/dl	8.4 - 10.2
Method: Arsenazo III			
Machine: Abbott ALINITY CI			
Serum Phosphorus	4.2	mg/dl	2.3 - 4.7
Method: Phosphomolybdate			
Machine: Abbott ALINITY CI			
Serum Sodium	139	mmol/L	136 - 145
Machine: Abbott ALINITY CI			
Serum Chloride	104	mmol/L	98 - 107
Method: ISE (Indirect)			
Machine: Abbott ALINITY CI			
Blood Urea	18	mg/dl	19.2 -45
Method: Urease			
Machine: Abbott ALINITY CI			
Blood Urea Nitrogen (BUN)	8.4	mg/dl	8.9 - 20.6
Method: Urease			
Bun/Creatinine Ratio	8.17	Ratio	
Method: Calculated			
Urea/Creatinine Ratio	17.48	Ratio	
Method: Calculated			

The kidneys play a vital role in the excretion of waste products and toxins such as urea, creatinine and uric acid, regulation of extracellular fluid volume, serum osmolality and electrolyte concentrations, as well as the production of hormones like erythropoietin and 1,25 dihydroxy vitamin D and renin. Assessment of renal function is important in the management of patients with kidney disease or pathologies affecting renal function. Tests of renal function have utility in identifying the presence of renal disease, monitoring the response of kidneys to treatment, and determining the progression of renal disease.

Urea is synthesized in the liver as the final product of protein and amino acid metabolism. Urea synthesis is therefore dependent on daily protein intake and endogenous protein metabolism.

Creatinine is a metabolic product of creatine and phosphocreatine, which are both found almost exclusively in muscle.

Uric Acid is the major product of purine catabolism in humans. Uric acid levels are used to monitor the treatment of gout.

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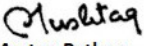
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Sample Type	: SERUM	Report Status	: Final Report	

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Test Name	Value	Unit	Bio. Ref Interval
Measurement of calcium is used in the diagnosis and treatment of parathyroid disease, a variety of bone diseases, chronic renal disease, urolithiasis and tetany. Phosphorus levels are increased in acute or chronic renal failure with decreased GFR . Sodium is an electrolyte, and it helps regulate the amount of water in and around the cells & the balance of chemicals in the body called acids and bases. Chloride is a negatively charged ion that works with other electrolytes such as potassium, sodium, and bicarbonate, to help regulate the amount of fluid in the body and maintain the acid-base balance.			


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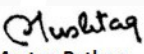
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Sample Type	: SERUM	Report Status	: Final Report	

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Test Name	Value	Unit	Bio. Ref Interval
Lipid Profile			
Total Cholesterol	144.5	mg/dl	Desirable : <200 Borderline: 200-239 High : >=240
Method: Enzymatic Machine: Abbott ALINITY CI			
Serum Triglycerides	70.0	mg/dl	Desirable : <150 Borderline high : 150-199 High : 200-499 Very high : > 500
Method: GPO-POD Machine: Abbott ALINITY CI			
Serum HDL Cholesterol	50.8	mg/dl	40 - 60
Method: Accelerator Selective Detergent Machine: Abbott ALINITY CI			
LDL Cholesterol Calculated	79.68	mg/dl	Optimal : <100 near /above Optimal:100 - 129 Borderline High: 130- 159 High : 160 - 189 Very High :>=190
Method: Calculated			
VLDL Cholesterol Calculated	14	mg/dl	<30
Method: Calculated			
Total CHOL / HDL Cholesterol Ratio	2.84	Ratio	3.30 - 4.40
Method: Calculated			
LDL / HDL Cholesterol Ratio	1.57	Ratio	Desirable/Low Risk: 0.5-3.0 Line/Moderate Risk: 3.0-6.0 Elevated/High Risk: >6.0
Method: Calculated			
HDL / LDL Cholesterol Ratio	0.64	Ratio	Optimal->0.4 Moderate-0.4 to 0.3 High-<0.3
Method: Calculated			
Non-HDL Cholesterol	93.7	mg/dl	0.0 - 160.0
Method: Calculated			

Dyslipidemia is a disorder of fat or lipoprotein metabolism in the body and includes lipoprotein overproduction or deficiency. Dyslipidemias means increase in the level of one or more of the following: Total Cholesterol, low density lipoprotein (LDL) and/or triglyceride concentrations. Dyslipidemia also includes a decrease in the "good" cholesterol or high-density lipoprotein (HDL) concentration in the blood. Cholesterol is a steroid carried in the bloodstream as lipoprotein, necessary for cell membrane functioning and as a precursor to bile acids, progesterone ,vitamin D ,estrogens ,glucocorticoids and mineralocorticoids.


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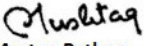
DEPARTMENT OF BIOCHEMISTRY

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Test Name	Value	Unit	Bio. Ref Interval
HDL is termed "good cholesterol" because its levels are inversely related to the risk of Coronary heart disease.			
LDL cholesterol is termed the "bad cholesterol" and their increased levels are associated with increased risk of atherosclerosis and coronary heart disease.			
Lipid level assessments must be made following 9 to 12 hours of fasting, otherwise assay results might lead to erroneous interpretation. Healthians labs report biological reference intervals (normal ranges) in accordance with the recommendations of The National Cholesterol Education Program (NCEP) & Adult Treatment Panel IV (ATP IV) guidelines providing the most desirable targets of various circulating lipid fractions in the blood. NCEP recommends that all adults above 20 years of age must be screened for abnormal lipid levels.			



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Sample Type	: URINE	Report Status	: Final Report

DEPARTMENT OF CLINICAL PATHOLOGY

Healthy India 2025 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
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Urine Routine & Microscopy Extended

PHYSICAL EXAMINATION

Colour	Pale Yellow		Pale Yellow
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Method: Visual

Volume	25.00	mL	
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Method: Visual

Appearance	Clear		Clear
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Method: Visual

CHEMICAL EXAMINATION

Specific Gravity	1.030		1.001 - 1.035
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Method: Dipstick-Ion exchange

Machine: Transasia Laura

pH	6.0		4.5 - 7.5
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Method: Dipstick-Double indicator

Machine: Transasia Laura

Glucose	Negative		Negative
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Method: Dipstick-oxidase peroxidase

Machine: Transasia Laura

Urine Protein	Negative		Negative
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Method: Dipstick-Bromophenol blue

Machine: Transasia Laura

Ketones	Negative		Negative
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Method: Sodium nitroprusside

Machine: Transasia Laura

Urobilinogen	Normal		Normal
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Method: Dipstick-Ehrlichs Test

Machine: Transasia Laura

Bilirubin	Negative		Negative
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Method: Dipstick-Ehrlichs Test

Machine: Transasia Laura

Nitrite	Negative		Negative
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Method: Dipstick-Griess test

Machine: Transasia Laura

Blood	Negative		Nil
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Method: Dipstick-Peroxidase

Machine: Transasia Laura

Mushtaq

Dr. Mustaq Pathan
MBBS, DCP, Pathologist
Reg. No: G-29980, Healthians Labs



SIN No: E3959015

Patient Name	: Radheshyam B Thakkar	Barcode	: E3959015	
Age/Gender	: 42Y 0M 0D /Male	Sample Collected On	: 01/Nov/2025 10:34AM	
Order Id	: 15711877853	Sample Received On	: 01/Nov/2025 03:15PM	
Referred By	: Self	Report Generated On	: 01/Nov/2025 04:38PM	
Customer Since	: 01/Nov/2025	Sample Temperature	: Maintained ✓	
Sample Type	: URINE	Report Status	: Final Report	

DEPARTMENT OF CLINICAL PATHOLOGY

Healthy India 2025 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
Leucocyte Esterase	Negative		Nil
Method: Dipstick- Esterase			
Machine: Transasia Laura			
MICROSCOPIC EXAMINATION			
Pus Cells	1-2	/HPF	0 - 5
Method: Microscopic Examination			
Epithelial cells	1-2	/HPF	0 - 5
Method: Microscopic Examination			
RBCs	Nil	/HPF	Nil
Method: Microscopic Examination			
Casts	Nil		Nil
Method: Microscopic Examination			
Crystals	Nil		Nil
Method: Microscopic Examination			
Bacteria	Absent		Absent
Method: Microscopic Examination			
Yeast Cell	Absent		Absent

The main indication for testing for glucose in urine is detection of unsuspected diabetes mellitus or follow-up of known diabetic patients. Renal glycosuria accounts for 5% of cases of glycosuria in general population.

Proteinuria can be seen in nephrotic syndrome, pyelonephritis, heavy metal poisoning, tuberculosis of kidney, interstitial nephritis, cystinosis, Fanconi syndrome, rejection of kidney transplant. Hemodynamic proteinuria is transient and can be seen in high fever, hypertension, heavy exercise, congestive cardiac failure, seizures, and exposure to cold. Post-renal proteinuria is caused by inflammatory or neoplastic conditions in renal pelvis, ureter, bladder, prostate, or urethra. Ketouria can be seen in uncontrolled Diabetes mellitus with ketoacidosis, Glycogen storage disorder, starvation, persistent vomiting in children, weight reduction program, fever in children, severe thyrotoxicosis, pregnancy and protein calorie malnutrition.

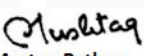
Presence of bilirubin in urine indicates conjugated hyperbilirubinemia (obstructive or hepatocellular jaundice). Bile salts along with bilirubin can be detected in urine in cases of obstructive jaundice. Normally about 0.5-4 mg of urobilinogen is excreted in urine in 24 hours. Therefore, a small amount of urobilinogen is normally detectable in urine. Increased urobilinogen in urine can be seen due to hemolysis, megaloblastic anemia and haemorrhage in tissues. Decreased urobilinogen can be seen in obstructive jaundice, reduction of intestinal bacterial flora, neonates and following antibiotic treatment. The presence of abnormal number of intact red blood cells in urine is called as hematuria. It implies presence of a bleeding lesion in the urinary tract. Hematuria can be seen in glomerular diseases like Glomerulonephritis, Berger's disease, lupus nephritis, Henoch-Schonlein purpura, non glomerular diseases like Calculus, tumor, infection, tuberculosis, pyelonephritis, hydronephrosis, polycystic kidney disease, trauma, after strenuous physical exercise, diseases of prostate (benign hyperplasia of prostate, carcinoma of prostate).

Nitrites are not present in normal urine. Ingested nitrites are converted to nitrate and excreted

in urine. If gram-negative bacteria (e.g. E.coli, Salmonella, Proteus, Klebsiella, etc.) are present in urine, they will reduce the nitrates to nitrites through the action of bacterial enzyme nitrate reductase. As E. coli is the commonest organism causing urinary tract infection, this test is helpful as a screening test for urinary tract infection.

Some organisms like Staphylococci or Pseudomonas do not reduce nitrate to nitrite and therefore in such infections nitrite test is negative.

Leucocyte esterase test detects esterase enzyme released in urine from granules of leucocytes. Thus the test is positive in pyuria.


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SIN No:E3959015

Patient Name	: Radheshyam B Thakkar	Barcode	: E3959015
Age/Gender	: 42Y 0M 0D /Male	Sample Collected On	: 01/Nov/2025 10:34AM
Order Id	: 15711877853	Sample Received On	: 01/Nov/2025 03:37PM
Referred By	: Self	Report Generated On	: 01/Nov/2025 05:21PM
Customer Since	: 01/Nov/2025	Sample Temperature	: Maintained ✓
Sample Type	: WHOLE BLOOD EDTA	Report Status	: Final Report

DEPARTMENT OF HAEMATOLOGY

Healthy India 2025 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
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Complete Blood Count Advanced

Haemoglobin (HB)	8.7	g/dL	13.0-17.0
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Method: Spectrophotometry

Machine: Yumizen H550E

Total Leucocyte Count (TLC)	13.9	10 ³ /uL	4.0-10.0
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Method: Impedance

Machine: Yumizen H550E

Result reconfirmed on peripheral smear. Kindly Correlate clinically.

Hematocrit (PCV)	28.7	%	40.0-50.0
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Method: Calculated

Machine: Yumizen H550E

Red Blood Cell Count (RBC)	4.58	10 ⁶ /μl	4.50-5.50
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Method: Impedance

Machine: Yumizen H550E

Mean Corp Volume (MCV)	62.7	fL	83.0-101.0
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Method: Derived from RBC Histogram

Machine: Yumizen H550E

Mean Corp Hb (MCH)	18.9	pg	27.0-32.0
--------------------	------	----	-----------

Method: Calculated

Machine: Yumizen H550E

Mean Corp Hb Conc (MCHC)	30.1	g/dL	31.5-34.5
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Method: Calculated

Machine: Yumizen H550E

RDW - CV	16.7	%	11.6-14.0
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Method: Derived from RBC Histogram

Machine: Yumizen H550E

RDW - SD	33.60	fL	39.0-46.0
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Method: Derived from RBC Histogram

Machine: Yumizen H550E

Mentzer Index	13.69	Ratio	
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Method: Calculated

RDWI	228.62	Ratio	
------	--------	-------	--

Method: Calculated

Green and king index	75	Ratio	
----------------------	----	-------	--

Method: Calculated

Differential Leucocyte Count

Mustaq

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SIN No: E3959015

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Sample Type	: WHOLE BLOOD EDTA	Report Status	: Final Report

DEPARTMENT OF HAEMATOLOGY

Healthy India 2025 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
Neutrophils	53.0	%	40 - 80
Method: Double Hydrodynamic Sequential System DHSS			
Machine: Yumizen H550E			
Lymphocytes	37	%	20-40
Method: Double Hydrodynamic Sequential System DHSS			
Machine: Yumizen H550E			
Monocytes	7.0	%	02 - 10
Method: Double Hydrodynamic Sequential System DHSS			
Machine: Yumizen H550E			
Eosinophils	3.0	%	01 - 06
Method: Double Hydrodynamic Sequential System DHSS			
Machine: Yumizen H550E			
Basophils	0.0	%	00 - 02
Method: Double Hydrodynamic Sequential System DHSS			
Machine: Yumizen H550E			
Absolute Leucocyte Count			
Absolute Neutrophil Count (ANC)	7.36	10 ³ /uL	2.0-7.0
Method: Calculated			
Machine: Yumizen H550E			
Absolute Lymphocyte Count (ALC)	5.14	10 ³ /uL	1.0-3.0
Method: Calculated			
Machine: Yumizen H550E			
Absolute Monocyte Count	0.97	10 ³ /uL	0.2-1.0
Method: Calculated			
Machine: Yumizen H550E			
Absolute Eosinophil Count (AEC)	0.42	10 ³ /uL	0.02-0.5
Method: Calculated			
Machine: Yumizen H550E			
Platelet Count(PLT)	819	10 ³ /μl	150-410
Method: Impedance			
Machine: Yumizen H550E			
Result reconfirmed on peripheral smear.Kindly Correlate clinically.			
MPV	6.1	fL	7 - 9
Method: Derived from PLT Histogram			
Machine: Yumizen H550E			
NLR (Neutrophil Lymphocyte Ratio)	1.43	Ratio	
Method: Calculated			
PLR (Platelet Lymphocyte Ratio)	159	Ratio	

Mustaq
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DEPARTMENT OF HAEMATOLOGY

Healthy India 2025 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
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Method: Calculated

The International Council for Standardization in Haematology (ICSH) recommends reporting of absolute counts of various WBC subsets for clinical decision making. This test has been performed on a fully automated 5 part differential cell counter which counts over 10,000 WBCs to derive differential counts. A complete blood count is a blood panel that gives information about the cells in a patient's blood, such as the cell count for each cell type and the concentrations of Hemoglobin and platelets. The cells that circulate in the bloodstream are generally divided into three types: white blood cells (leukocytes), red blood cells (erythrocytes), and platelets (thrombocytes). Abnormally high or low counts may be physiological or may indicate disease conditions, and hence need to be interpreted clinically.

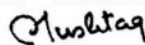
The Mentzer index is used to differentiate iron deficiency anaemia beta thalassemia trait. If a CBC indicates microcytic anaemia, these are two of the most likely causes, making it necessary to distinguish between them.

If the quotient of the mean corpuscular volume divided by the red blood cell count is then 13, thalassemia is more likely. If the result is greater than 13, then iron-deficiency anaemia is more likely.

Green and King Index used to differentiate IDA from thalassemia trait value >65 is likely to be Iron Deficiency Anemia and value <65 Beta Thalassemia Trait. For RDW Value >220 more likely to be Iron Deficiency Anemia and value <220 more likely to be Beta Thalassemia Trait.

The neutrophil to lymphocyte ratio (NLR) is considered to be a sensitive indicator of infection, inflammation and sepsis, and a robust prognostic marker of disease severity.

The platelet-to-lymphocyte ratio (PLR) is a marker revealing shifts in platelet & lymphocyte cell counts due to acute inflammatory and pro-thrombotic states.



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Age/Gender	: 42Y 0M 0D /Male	Sample Collected On	: 01/Nov/2025 10:34AM	
Order Id	: 15711877853	Sample Received On	: 01/Nov/2025 03:42PM	
Referred By	: Self	Report Generated On	: 01/Nov/2025 04:29PM	
Customer Since	: 01/Nov/2025	Sample Temperature	: Maintained ✓	
Sample Type	: Serum	Report Status	: Final Report	

DEPARTMENT OF IMMUNOLOGY

Healthy India 2025 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
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Vitamin B12

VITAMIN B12	316	pg/ml	211 - 911
Method: CLIA			
Machine: Siemens Centaur XP			

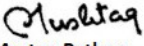
Vitamin B12 is a coenzyme that is involved in two very important metabolic functions vital to normal cell growth and DNA synthesis: 1) the synthesis of methionine, and 2) the conversion of methylmalonyl CoA to succinyl CoA. Deficiency of this vitamin can lead to megaloblastic anemia and ultimately to severe neurological problems. Also causes macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination, and affective behavioral changes. A significant increase in RBC MCV may be an important indicator of vitamin B12 deficiency. Patients taking vitamin B12 supplementation may have misleading results. A normal serum concentration of B12 does not rule out tissue deficiency of vitamin B12. The most sensitive test for B12 deficiency at the cellular level is the assay for MMA. If clinical symptoms suggest deficiency, measurement of MMA and homocysteine should be considered, even if serum B12 concentrations are normal.

Vitamin D, 25-Hydroxy

VITAMIN D (25 - OH VITAMIN D)	28.75	ng/ml	30 - 100
Method: CLIA			
Machine: Siemens Centaur XP			

VITAMIN D STATUS	VITAMIN D 25 HYDROXY (ng/mL), Adult	VITAMIN D 25 HYDROXY (ng/mL), Pediatric
DEFICIENCY	<20	<15
INSUFFICIENCY	20 - 30	15 - 20
SUFFICIENCY	30 - 100	20 - 100

Vitamin D is a lipid-soluble steroid hormone that is produced in the skin through the action of sunlight or is obtained from dietary sources. The role of vitamin D in maintaining homeostasis of calcium and phosphorus is well established. The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D. Vitamin D status is best determined by measurement of 25 hydroxy vitamin D, as it is the major circulating form and has longer half life (2-3 weeks) than 1,25 Dihydroxy vitamin D (5-8 hrs). The reference ranges discussed in the preceding are related to total 25-OHD; as long as the combined total is 30 ng/mL or more, the patient has sufficient vitamin D. Levels needed to prevent rickets and osteomalacia (15 ng/mL) are lower than those that dramatically suppress parathyroid hormone levels (20-30 ng/mL). In turn, those levels are lower than levels needed to optimize intestinal calcium absorption (34 ng/mL). Neuromuscular peak performance is associated with levels approximately 38 ng/mL.


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Sample Type	: Serum	Report Status	: Final Report

DEPARTMENT OF IMMUNOLOGY

Healthy India 2025 Full Body Checkup Prime

Test Name	Value	Unit	Bio. Ref Interval
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Thyroid Profile (Total T3,T4, TSH)

Tri-Iodothyronine (T3, Total)	0.83	ng/ml	0.6 - 1.81
Method: CLIA Machine: Siemens Centaur XP			
Thyroxine (T4, Total)	5.60	ug/dl	3.2-12.6
Method: CLIA Machine: Siemens Centaur XP			
Thyroid Stimulating Hormone (TSH)-Ultrasensitive	2.031	μIU/ml	0.35 - 5.50
Method: CLIA Machine: Siemens Centaur XP			

Pregnancy interval	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 - 3.0
Third trimester	0.3 - 3.0

Healthians recommends that the following potential sources of variation should be considered while interpreting thyroid hormone results:

1. Thyroid hormones undergo rhythmic variation within the body this is called circadian variation in TSH secretion: Peak levels are seen between 2-4 AM. Minimum levels seen between 6-10 AM. This variation may be as much as 50% thus, influence of sampling time needs to be considered for clinical interpretation.
2. Circulating forms of T3 and T4 are mostly reversibly bound with Thyroxine binding globulins (TBG), and to a lesser extent with albumin and Thyroid binding Pre-Albumin. Thus the conditions in which TBG and protein levels alter such as chronic liver disorders, pregnancy, excess of estrogens, androgens, anabolic steroids and glucocorticoids may cause misleading total T3, total T4 and TSH interpretations.
3. Total T3 and T4 levels are seen to have physiological rise during pregnancy and in patients on steroid treatment.
4. T4 may be normal even in the presence of hyperthyroidism under the following conditions : T3 thyrotoxicosis, Hypoproteinemia related reduced binding, during intake of certain drugs (eg Phenytoin, Salicylates etc)
5. Neonates and infants have higher levels of T4 due to increased concentration of TBG
6. TSH levels may be normal in central hypothyroidism, recent rapid correction of hypothyroidism or hyperthyroidism, pregnancy, phenytoin therapy etc.
7. TSH values of <0.03 uIU/mL must be clinically correlated to evaluate the presence of a rare TSH variant in certain individuals which is undetectable by conventional methods.
8. Presence of Autoimmune disorders may lead to spurious results of thyroid hormones.
9. Various drugs influence the levels of thyroid hormones such as L-Dopa, Lithium, Glucocorticoids, Phenytoin etc.
10. Healthians recommends evaluation of unbound fractions, that is free T3 (fT3) and free T4 (fT4) for clinic-pathologic correlation, as these are the metabolically active forms.

*** End Of Report ***

Mushtaq
Dr. Mushtaq Pathan
MBBS, DCP, Pathologist
Reg. No: G-29980, Healthians Labs

SIN No:E3959015

Terms & Conditions:

- 1) Machine Data is available for last 7 days only. In case of manual testing & outsourced testing, machine data will not be available.
- 2) CBC parameters may vary when it is manually reviewed by the Pathologists.
- 3) **For Thyroid tests** - Circulating TSH shows a normal circadian rhythm with a peak between 11pm-5am and a nadir between 5pm-8pm. TSH values are also lowered after food when compared to fasting in a statistically significant manner. This variation is of the order of $\pm 50\%$, hence time of day and fasting status have influence on the reported TSH level.
- 4) **For Lipid profile** - Lipid and Lipoprotein concentrations vary during the normal course of daily activity. Also, certain drugs, diet and alcohol can have lasting effects on Triglyceride levels. To obtain best results for Lipid testing, a strict fasting of 10-12 hours with a light meal on the previous night is recommended.
- 5) Test results released pertain to the specimen submitted.
- 6) Test results are dependent on the quality of the sample received by the Lab.
- 7) The tests are carried out in the lab with the presumption that the specimen belongs to the patient named or identified in the bill/test request form/booking ID.
- 8) The reported results are for information and are subject to confirmation and interpretation by the referring doctor to co-relate clinically.
- 9) Test results may show interlaboratory variations.
- 10) Liability of Healthians for deficiency of services or other errors and omissions shall be limited to the fee paid by the patient for the relevant laboratory services.
- 11) This report is not subject to use for any medico-legal purposes.
- 12) Few of the tests might be outsourced to partner labs as and when required.
- 13) This report is not intended to replace but to lead by providing comprehensive information. It is recommended that you consult your doctor/physician for interpretation of results.
- 14) All reports might not be applicable for individuals less than 18, pregnant women or individuals suffering from diseases for which health test has not been performed or symptoms not diagnosed.
- 15) This report is based on preventive health test screening and is meant for a healthy lifestyle. It does not provide any recommendation for life threatening situations.
- 16) It is strongly recommended to take required precautions for allergic reactions or sensitivities.
- 17) Authorised partner labs as mentioned for certain tests are as below:
 - HL/PL/001- Metropolis Healthcare Ltd
 - HL/PL/002- Thyrocare technologies Limited
 - HL/PL/003- Lifecell International Pvt. Ltd. - Laboratory Services
 - HL/PL/004- Modern Diagnostic & Research Centre

ADVISORY

Health Advisory

Radheshyam B Thakkar

Booking ID : 15711877853 | Sample Collection Date : 01/Nov/2025

27.61 Body Mass Index

5'5" Height (ft/in)

77 Weight (kgs.)



Physical Activity

No Data



Smoke

No Data



Food Preference

No Data



Alcohol

No Data



Medication

No Data



Family History

No Data



Blood Pressure

No Data



Pulse Rate

No Data



Waist (In Cm)

No Data



Hip Circumference (In Cm)

No Data



SPO2 Levels

No Data



Sugar Levels

No Data

Additional Remarks :

NA

SUGGESTED
NUTRITION

SUGGESTED NUTRITION

Do's

- Include seeds like flaxseeds, chia seeds, sunflower seeds
- Include fruits like apples, berries and melons in your diet
- Have a balanced diet that includes whole grains, pulses, dairy, fruits, vegetables, nuts and healthy fats
- Have dates and figs
- Take vitamin C rich foods like citrus fruits, strawberries and green, leafy vegetables
- Include calcium rich foods like milk, yoghurt, cheese and green, leafy vegetables
- Include Brazil nuts, sesame seeds, sunflower seeds
- Take calcium fortified cereals, breads and juices
- Take fresh fruit and vegetable juices

Dont's

- Limit sugar intake
- Limit tea and coffee
- Decrease intake of colas and sugary drinks
- Avoid refined carbs, processed foods
- Avoid the use of oil and avoid sauces and dressings
- Reduce caffeine intake
- Avoid flavoured and seasoned foods
- Avoid saturated fats, trans fats, oily and greasy foods like cakes, creamy or fried foods
- Avoid red meat and organ meats

SUGGESTED
LIFESTYLE

SUGGESTED LIFESTYLE

Do's

- Lose weight gradually and stay active
- Maintain ideal weight
- Have regular exposure to sunlight
- Stay active and maintain ideal weight

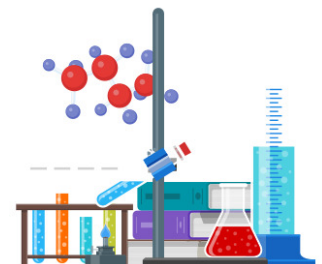
Dont's

- Avoid late night heavy meals
- Avoid overworking or being stressed for long time
- Avoid smoking and alcohol
- Avoid overexertion without having food or drink
- Avoid strenuous exercises
- Avoid long periods of inactivity
- Limit dining out

SUGGESTED
FUTURE
TESTS

SUGGESTED FUTURE TESTS

- Glycated Hemoglobin (HbA1c) - Every 3 Month
- Blood Glucose Fasting - Every 1 Week
- Glucose Postprandial - Every 1 Week
- Complete Hemogram - Every 1 Month
- Iron Studies With Ferritin - Every 1 Month
- Occult blood, Stool - Every 1 Month
- Reticulocyte Count - Every 1 Month



HEALTH ADVISORY

Suggestions for Health & Well-being

Radheshyam B Thakkar

Booking ID : 15711877853 | Sample Collection Date : 01/Nov/2025

PHYSICAL ACTIVITY

PHYSICAL ACTIVITY

Physical activities can vary from Regular walks (Brisk or normal), Jogging , Sports, Stretching, Yoga to light weight lifting etc. It is recommended to partake in physical activity at least 30 minutes a day for 3-4 days a week.

If regular workout is difficult, then we can adapt changes such as using stairs instead of lift/escalators and doing household work!



BALANCED DIET

A balanced diet is the key to healthy lifestyle. Include Whole grains, vegetables, whole fruits, nuts, seeds, beans, plant oils in your diet.

It is recommended to always have a high protein breakfast and a light dinner. Avoid items such as processed foods, potatoes and high calorie/sugar products. Don't forget to drink water regularly!

BALANCED DIET

STRESS MANAGEMENT

STRESS MANAGEMENT

Managing stress is an essential part of well-being. Some day to day changes can help such as having sufficient sleep (6-8 hours), indulging yourself in meditation, positive attitude towards lifestyle, using humor, traveling, talking to people whom you feel comfortable with and making time for hobbies by doing what you love to do.



BMI

BMI recommended range is 18.5 to 24.9. Your BMI is 29.35, which is on a higher side.

BMI INFORMATION NOT AVAILABLE
Please fill your Health Karma to know your BMI results
BMI for your body helps prevent many untimely diseases and goes a long way.

BMI CHART

UNDERWEIGHT	NORMAL	OVERWEIGHT	OBSE
Less than 18.5	Between 18.5 - 24.9	Between 25.0 - 29.9	More than 30
*****	*****	*****	*****

BMI

Supplement Suggestions

Radheshyam B Thakkar

Booking ID : 15711877853 | Sample Collection Date : 01/Nov/2025

Your test report has indicated that you have certain deficiencies in your body which may hamper your health & wellbeing in the longer run.

In order to fulfill the gaps in nutrition and promote a healthier body we suggest you the following supplements mentioned below:

Deficiency/Out of Range Parameter(s)	Suggested Supplement
Glycated Hemoglobin (HbA1c)	DIABEAT-EASE
Iron, Serum	IRON POWER

To order, call 1800-572-000-4

Suggestions for Improving Deficiencies



DIABEAT-EASE

Manage diabetes the all-natural way!

An all-natural supplement that helps in lowering your blood sugar levels, thus preventing the onset of diabetes and managing it if you are already a diabetic. By reducing blood sugar levels, this naturally-sourced diabetes supplement enables you to lead a productive life, while managing your diabetes in a safe and natural way.

Remember, ignoring diabetes can cause a lot of serious complications, including:

- Vision & Hearing Loss
- Nerve Damage
- Heart Attack
- Stroke
- Dementia

Infused with the ages-proven goodness of all-natural ingredients, DIABEAT-EASE is the perfect supplement to help you control diabetes without having to worry about side-effects. Sourced from nature's own pharmacy of herbs, the ingredients in DIABEAT-EASE present the following benefits:

Saunf

Helps control blood pressure & manage diabetes

Karela

Reduces blood sugar & reduces cholesterol levels

Chirata

Helps manage high blood pressure, diabetes & detoxifies blood

Ashwagandha

Reduces blood sugar, cholesterol, & triglycerides levels

Vijayasar

Helps manage diabetes by lowering down sugar cravings



IRON POWER

Your all natural weapon against Anaemia

IRON-POWER is a scientifically formulated and clinically proven all-natural supplement that helps you replenish iron in your system and significantly reduce the risk of anemia. This ayurvedic supplement increases blood production and iron levels to keep your body functioning optimally. Keep overall health robust and treat iron deficiency with IRON-POWER

It's widely known that iron is crucial for your health. A lack of iron intake or iron deficiency can cause serious health issues, such as:

- Heart problems
- Growth issues in children
- Pregnancy complications
- Depression
- Increased infection risk

Infused with the ages-proven goodness of all-natural ingredients, IRON-POWER is the perfect supplement to enhance and maintain your iron levels without having to worry about side effects. Sourced from nature's own pharmacy of herbs, the ingredients in IRON-POWER present the following benefits:

Orange Peel Extract

Rich in vitamin C. Helps maintain iron levels

Supplement Suggestions

Radheshyam B Thakkar

Booking ID : 15711877853 | Sample Collection Date : 01/Nov/2025

Deficiency/Out of Range Parameter(s)	Suggested Supplement
Vitamin D Total-25 Hydroxy	VITAMIN D3

To order, call 1800-572-000-4

Suggestions for Improving Deficiencies



VITAMIN D3

Improve bone health with enhanced calcium absorption, the natural way

Make your muscles and bones stronger with VITAMIN D3. Sourced from natural substances, it helps in regulating the absorption of calcium and phosphorus, which help keep your bones strong and enhancing the normal immune system functioning. Vitamin D3 is an essential nutrient that's critical for normal growth and development of bones and teeth, as well as improved resistance against certain diseases.

Remember, a lack of vitamin D3 can cause dangerous health situations.

- Rickets (in children) | • Brittle Bones | • Osteoporosis | • Weakened Bones (in adults)

Strengthens Bones & Muscles

Protects Against Pneumonia & Acute Respiratory Infections

Helps in Reducing Depression

Boosts Heart Health

Aids in Kidney Disease Treatment



IMMUNO-PLUS

Give your immunity a boost the all-natural way.

IMMUNO-PLUS is the perfect all-natural herbal supplement to boost your immune system and strengthens your body's defenses against diseases and infections. IMMUNO-PLUS provides your immune system the necessary reinforcement to keep you safe and healthy.

A weakened immune system opens you to a host of illnesses, such as:

- Recurring Infections | • Heightened Risk of Cancer | • Autoimmune Disorders | • Slow Growth Rate | • Serious Damage to the Heart, Lungs, Digestive Tract & the Nervous System

Infused with the ages-proven goodness of all-natural ingredients, IMMUNO-PLUS is the perfect supplement to strengthen your immune system without having to worry about side effects. Sourced from nature's own pharmacy of herbs, the ingredients in IMMUNO-PLUS present the following benefits:

Amla

Boosts immunity & Stores antioxidants

Jetwatika

Antioxidant properties strengthen the immune system

Aloe Vera

Fights against oxygenated rogue molecules in the blood

Ashwagandha

Reinforces the immune system to increase its fighting ability

Ginger

Anti-inflammatory & antioxidant effects reinforce the immune system



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Know More

About Healthians Labs

How we control Report Accuracy at Healthians



Quality Control

We follow Quality control to ensure both **precision & accuracy** of patient results.



Machine Data

We save patient's result values **directly from machines** ensuring no manipulations & no fake values.



QR Code

QR Code based authenticity check on all its reports



Calibration

We make use of calibrators to evaluate the **precision & accuracy** of measurement equipment.



Equipment

Our Labs are equipped with state-of-the-art instruments with **cutting edge technology** to provide faster & reliable results.



EQA

Our Labs participate in EQA & show proven accuracy by checking **laboratory performance** through external agency or facility.

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